

ClinicalHours

Business Plan

Shivam Kanodia | Raghav Tirupati

clinicalhours.org | Texas A&M University | 2025

1. Executive Summary

ClinicalHours is a two-sided marketplace connecting pre-med students with clinical volunteering opportunities at hospitals and clinics. Students find, apply for, and track their clinical hours in one place. Clinics manage applications, communicate with candidates, and coordinate scheduling from a single dashboard.

We launched in January 2025 and reached 200 organic users in under three months with no paid marketing. Our first clinic partner, BCS Free Health Clinic, is live. We are accepted into the Meloy Launch Accelerator and placed in the Good Bull Pitch (3rd) and Ideas Challenge (Finalist).

We are seeking \$10,000 in non-dilutive funding to formalize legal infrastructure, build our iOS app, and accelerate clinic and student acquisition across Texas.

200+ Users	2 Clinic Partners	3 Competitions	\$10K Ask
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2. Problem and Solution

The Problem

Pre-med students need clinical volunteer hours to apply to medical school. Most need 100+ hours and spend weeks cold-calling hospitals, sending mass emails, and maintaining personal spreadsheets to track outreach. Many walk into hospitals uninvited because there is no better system.

On the clinic side, hospitals and free clinics receive disorganized waves of student inquiries, especially during peak application season. Most manage this through email and spreadsheets. There is no tool built specifically for this workflow.

Neither side has infrastructure. Both sides waste hours on a process that should take minutes.

The Solution

ClinicalHours gives each side exactly what it needs.

- Students browse verified opportunities, apply directly, track hours, and log reflections. Six AI tools assist with AMCAS activity writing, AAMC competency tracking, and secondary essays.
- Clinics post positions, manage a pipeline of applicants with custom application forms and filters, send bulk emails, and schedule interviews from one dashboard.

The product is live at clinicalhours.org. Tech stack: Next.js, Supabase, Vercel. Gmail OAuth is integrated for clinic email. A four-step student application flow and six-page admin portal are fully deployed.

3. Market Opportunity

Market sizing uses AAMC, HRSA, AHA, NAFC, and Texas DSHS as primary sources. All clinic figures are per service delivery site, consistent with our per-site pricing model.

	Student Side	Clinic Side
TAM	191,000 serious pre-med students nationally. 5% premium conversion at \$60/yr = \$573K	32,380 clinic sites (hospitals, FQHC sites, free clinics, independent urgent cares) at \$750/site = \$24.3M. Combined TAM: ~\$24.9M
SAM	133,000 reachable students via ambassador program (950 universities, 2 orgs each, 70 active members). 5% conversion = \$399K	17,600 FQHC and free clinic sites with active volunteer programs. 30% lack structured management = 5,280 sites at \$750 = \$3.96M. Combined SAM: ~\$4.4M
SOM	18,900 serious Texas pre-med students. 5% premium conversion at \$60/yr = \$56,700	1,250 Texas clinic sites (700 FQHC sites + 550 free clinics). 60% willing to pay at \$750 = \$562,500. Combined SOM: ~\$619K

Sources: AAMC 2025, HRSA bphc.hrsa.gov, Texas DSHS dshs.texas.gov, NAFC nafcclinics.org, AHA 2024, Journal of Urgent Care Medicine 2024. SOM reflects market boundary only; sales execution assumptions are modeled in financial projections.

4. Business Model

Revenue Streams

Stream	Who Pays	Price	Notes
Student Premium	Pre-med students	\$4.99/month (\$60/yr)	Freemium. AI tools (AMCAS writer, competency tracker, essay helper) unlock at premium tier.
Clinic Subscription	Hospitals and clinics	\$750-1,000/site/yr	Custom portal, applicant sorting, bulk email, interview scheduling. Priced per service delivery site.
Advisor Dashboard	University pre-med offices	\$5,000/yr per office	Launches Y4. Advisors track student progress, manage placements, and communicate at scale.

Unit Economics

- Clinic CAC: Primarily outreach time. No paid acquisition in Y1. Cash cost per clinic is effectively \$0 in Y1.
- Student CAC: Commission-based ambassador model. No upfront cost. Paid influencer content at \$25/video for awareness.
- Clinic LTV: \$750-1,000/site/yr with 10% annual churn implies 8-10 year average relationship at full scale.
- Student LTV: High churn by design (one application cycle), but new cohorts replace churned users each year through university partnerships.

5. Traction and Milestones

Product

- Fully launched at clinicalhours.org in January 2025
- Full admin portal deployed: Overview, Applications, Interviews, Positions, Email, History
- Student four-step application flow live
- Gmail OAuth integrated for clinic admin email
- 200+ registered students, no paid marketing
- iOS app in development

Partnerships

- BCS Free Health Clinic: first live clinic partner
- Agape Clinic: second confirmed partner (Dora Gonzalez, decision-maker)
- Active outreach pipeline across BCS and DFW markets

Competition

- Good Bull Pitch: 3rd place, \$200
- Ideas Challenge: Finalist
- Meloy Launch Accelerator: Accepted (3 of 11 teams)
- Aggie Pitch: In preparation

Advisors

- William Shaw, Volunteer Manager at St. Joseph Health: validated the clinic-side problem; informed our pivot from hospital-focused to free clinic focus
- Mark Radulovich, Founder of Prota Financial: advises on pitching, market strategy, and financial modeling
- Rajeev Prashar, Enterprise Architecture and Strategy Leader at Ascension: confirmed large hospital systems have tools in place; validated free clinic focus; introduced the Athenahealth exit thesis

6. Go-to-Market Strategy

Clinic Acquisition (Supply Side)

We target free clinics and FQHCs first. These organizations are volunteer-dependent by design, have no existing management infrastructure, and are the most receptive to a \$750/year solution. Large hospital systems already have tools in place and are not our near-term target.

- Direct outreach: Shivam contacts clinic administrators directly with verified decision-maker contacts. Current pipeline covers BCS and DFW markets.
- Referral: Each clinic partner becomes a reference for the next. BCS Free Health Clinic opens doors to similar organizations across Texas.
- Conference presence: Pre-med and community health conferences as we scale regionally in Y3.

Student Acquisition (Demand Side)

Students are acquired through university pre-med organizations via a campus ambassador program. Each ambassador is commission-based, so acquisition cost scales only with revenue. Starting Y2, we supplement with paid ads targeting pre-med students on social platforms.

- Y1: 3 universities (TAMU, 2 DFW schools). Ambassador-driven, organic only.
- Y2: 10 Texas universities. Ambassador program + paid social ads (\$5,000).
- Y3+: Regional expansion to 30 universities. Paid ads scale to \$15,000 for regional reach.
- Supplementary: Pre-med micro-influencers on TikTok and YouTube at \$25/video throughout.

Y4 Expansion: Advisor Dashboard

By Y4 we have relationships at 75+ universities through the ambassador program. We leverage those to pitch the advisor dashboard directly to pre-med advising offices at \$5,000/year per office.

7. Competitive Landscape

No competitor owns both sides of the clinical volunteering marketplace. We sit in a gap that no existing platform addresses.

	ClinicalHours	VolunteerMatch	Handshake	Volgistics	Excel/Email
Pre-med specific	Yes	No	No	No	No
Student discovery	Yes	Partial	Yes	No	No
Clinic application portal	Yes	No	No	Yes	No
Hour tracking	Yes	No	No	Yes	No
AI tools for med school	Yes	No	No	No	No
Priced for small clinics	Yes	N/A	N/A	No	Free

Volgistics is our closest comparison on the clinic side. It is generic volunteer management software designed for large organizations, not built for pre-med students, and overpriced for smaller free clinics. Our moat is the network effect: every clinic we add makes the platform more valuable to students, and every student strengthens the case for the next clinic.

8. Team

Shivam Kanodia

Co-Founder, Product and GTM

Freshman, Industrial and Systems Engineering (Honors), Texas A&M University. Leads go-to-market strategy, clinic outreach, pitch competitions, and long-term product direction. Background in ML pipelines (XGBoost, LightGBM, SHAP), product thinking, and business research.

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Raghav Tirupati

Co-Founder, Product and Development

Pre-med student, Texas A&M University. Built ClinicalHours from the problem he lived firsthand, tracking hospital outreach on spreadsheets before founding the company. Leads feature development and leverages direct credibility with the student user base.

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Together, we represent both sides of the market. Raghav is the customer. Shivam builds the business around him. We went from idea to live product with 200 organic users in under three months.

9. Financial Projections

All projections follow assumptions in the accompanying financial model (ClinicalHours_Financial_Model_v3.xlsx). Costs are built bottom-up from actual line items with no salary assumptions. Both founders operate without compensation through Y5. Paid ads begin Y2 and scale with revenue growth.

Key Revenue Assumptions

- Clinic close rate grows from 1% (Y1, TX) to 10% (Y5, national) as network effects compound
- Student premium conversion held flat at 5%, consistent with freemium SaaS benchmarks
- Student churn is 60% annually by design: one application cycle per student, offset by new cohort each year
- Advisor dashboard launches Y4 at \$5,000/yr per office via warm intros through ambassador relationships
- Clinic price increases from \$750/site (Y1-Y2) to \$1,000/site (Y3+) as product matures

	Y1 (2025)	Y2 (2026)	Y3 (2027)	Y4 (2028)	Y5 (2029)
CLINIC REVENUE					
Active Clinic Sites	13	62	185	373	628
Price/Site (\$/yr)	\$750	\$750	\$1,000	\$1,000	\$1,000
Clinic Revenue	\$9,375	\$46,500	\$185,000	\$373,000	\$628,000
STUDENT REVENUE					
Universities	3	10	30	75	150
Premium Subscribers	3	11	33	86	183
Student Premium Revenue	\$180	\$660	\$1,980	\$5,160	\$10,980
ADVISOR DASHBOARD					
University Offices	0	0	0	3	12
Advisor Revenue	\$0	\$0	\$0	\$15,000	\$60,000
TOTAL REVENUE	\$9,555	\$47,160	\$186,980	\$393,160	\$698,980

Operating Cost Build

No salaries are included. Both founders operate without compensation through the projection period. Costs scale with product usage and marketing.

Line Item	Y1	Y2	Y3	Y4	Y5
Tools + APIs	\$1,640	\$2,000	\$4,000	\$8,000	\$12,000
Infrastructure	\$200	\$600	\$2,400	\$6,000	\$10,000
Influencer + Ambassador	\$500	\$1,250	\$5,000	\$8,000	\$12,000
Campus Outreach	\$300	\$1,500	\$0	\$0	\$0
Paid Ads	\$0	\$5,000	\$15,000	\$30,000	\$50,000

Events + Conferences	\$700	\$1,500	\$3,000	\$5,000	\$8,000
Legal	\$2,500	\$500	\$2,000	\$3,000	\$5,000
Dashboard Build	\$0	\$0	\$0	\$15,000	\$0
Misc	\$100	\$500	\$1,000	\$4,000	\$6,000
Total Costs	\$5,839	\$12,350	\$32,400	\$76,000	\$98,000

	Y1 (2025)	Y2 (2026)	Y3 (2027)	Y4 (2028)	Y5 (2029)
Total Revenue	\$9,555	\$47,160	\$186,980	\$393,160	\$698,980
Total Costs	\$5,839	\$12,350	\$32,400	\$76,000	\$98,000
NET INCOME	\$3,716	\$34,810	\$154,580	\$317,160	\$600,980

Note: Revenue figures are approximate; verify against the financial model which calculates all figures from sourced assumptions. Net income is positive from Y1 because both founders operate without compensation. This changes materially once salaries are introduced.

Path to Scale

With no salary burden, the business generates positive net income from Year 1. The primary cost driver is paid advertising, which scales with market expansion. The advisor dashboard build in Y4 (\$15,000 external dev) is the largest one-time cost and is funded by clinic subscription revenue by that stage.

10. Use of Funds

ClinicalHours is requesting \$10,000 in non-dilutive funding. Prior funding: \$200 prize from Good Bull Pitch. No other capital raised to date.

Category	Amount	Breakdown
Legal	\$2,500	Texas LLC formation, clinic B2B contract templates, privacy policy and terms of service via Clerky and Termly
Product	\$1,500	Apple Developer Program (\$99), OpenAI and Anthropic API costs, Vercel hosting, Lovable and Cursor subscriptions
Growth	\$4,800	Pre-med influencer content (\$25/video), campus outreach materials, pitch competition travel and registration, initial paid social ads
Operations	\$1,200	Domain, productivity tools, Claude Code subscription, miscellaneous
\$10,000 Total		6-month runway. Target: 1,000 students, 5 paying clinic sites.

6-Month Targets with This Funding

- Close 3-5 paying clinic sites across BCS and DFW
- Reach 1,000 registered students via ambassador program at TAMU and 2 DFW schools
- Launch iOS app
- Formalize LLC and B2B contract infrastructure
- Begin outreach to regional clinics outside Texas for Y2 expansion